

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction
for the administration of Shingrix for the prevention of Shingles

Summary of NICE and NICE CKS Guidelines for Shingles Vaccination

Purpose of Vaccination

- To **prevent shingles (herpes zoster)** and its complications, particularly **postherpetic neuralgia (PHN)**.
- Risk of shingles increases with **age** and **weakened immunity**.

Vaccines Available

The **shingles vaccine** currently in use in the UK is:

1. **Shingrix® (recombinant, non-live)**
 - **Preferred vaccine**, especially for immunocompromised individuals.
 - Suitable for people **aged 50 years and older** or **18+ if immunocompromised**.
 - Administered **intramuscularly**.

Eligibility (UK Immunisation Schedule)

- **Routine vaccination:**
 - All individuals **aged 65 years**, starting from 1 September 2023.
 - A **catch-up programme** includes those turning 66 to 70 years between 1 September 2023 and 31 August 2028.
- **Immunocompromised individuals:**
 - Aged **18 years and over** are eligible for **Shingrix®**.
 - Includes those with conditions such as leukaemia, HIV, or on immunosuppressive therapy.

Dosing Schedules

- **Shingrix®:**
 - **Two doses** given **2 to 6 months apart**.

Contraindications

- **Shingrix:**

- Hypersensitivity to any component.

Precautions

- Delay vaccination in cases of acute illness with fever.
- Not for treatment of acute shingles.
- Check vaccine history prior to administration.

Common Side Effects

- **Shingrix:** Localised pain, fatigue, headache, myalgia, fever.

Effectiveness

- **Shingrix:** Over **90% effective** at preventing shingles and PHN.

Documentation

- Record date, site, batch number, and brand of vaccine.
- Report any suspected adverse reactions to the **Yellow Card Scheme**.

Patient Group Direction

for the supply/administration of

Shingrix

for vaccination against

Shingles

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	• Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGD's to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Shingrix is indicated for the prevention of herpes zoster (shingles) and herpes zoster-related postherpetic neuralgia in individuals aged 50 years and older.
Inclusion criteria	- Individuals aged 50 years or older. - No previous shingles vaccination. - No history of shingles in the past 12 months. - Informed consent obtained. - Note that individuals may be eligible to receive the vaccine free of charge under the national immunisation programme. - The aim of the national shingles immunisation programme is to lower the incidence and severity of shingles in older people and those at increased risk. - From 1st September 2023, the JCVI recommended that the programme should be offered at 60 years of age. This is being rolled out over a period of years starting with those aged 65 and 70. Those who have been previously eligible will remain eligible until their 80th birthday. Where an individual has turned 80 years of age following their first dose of Shingrix, a second dose should be provided before the individual's 81st birthday to complete the course. The course consists of two doses of Shingrix which for operational reasons in the NHS has a dose interval of 6-12 months in England. Adults aged 70 to 79 years prior to 1st September 2023 will be eligible for vaccination until their 80th birthday. - From September 2025, Shingrix will also be offered to all severely immunosuppressed individuals aged 18 years and over (with no upper age limit). Eligible individuals should be offered two doses of Shingrix. For this cohort the second dose should be given 8 weeks to 6 months after the first dose as these individuals need to be protected more quickly. Note: - Shingrix can be given concomitantly with seasonal influenza vaccine (inactivated, unadjuvanted), 23 valent pneumococcal polysaccharide vaccine (PPV23), 13 valent pneumococcal conjugate vaccine (PCV13), reduced antigen diphtheria tetanus acellular pertussis vaccine (dTpa), coronavirus disease 2019 (COVID-19) messenger ribonucleic acid (mRNA) vaccine or respiratory syncytial virus (RSV) vaccine (recombinant, adjuvanted). The vaccines should be administered at different injection sites. - Initially, a seven-day interval was recommended between Shingrix® and adjuvanted influenza vaccines because of a concern that the potential reactogenicity from two adjuvanted vaccines might reduce tolerability in those being vaccinated. Interim data from a US study on co-administration of Shingrix® with adjuvanted seasonal influenza vaccines is however very reassuring. Therefore, an appointment for administration of the seasonal influenza vaccine can be an opportunity

	to also provide shingles vaccine, although the latter should be offered all year round, rather than purely as a seasonal programme.
Exclusion criteria	- Hypersensitivity to any component of the vaccine. - Pregnancy or breastfeeding (not routinely recommended). - Have current shingles infection with active lesions - Have already received one dose of Zostavax or two doses of Shingrix - Have received a dose of Shingrix in the last 8 weeks
Cautions	- Counsel patient that systemic side effects are common and generally self-limiting. - Allow appropriate spacing from other vaccines (e.g., COVID-19 or influenza) based on clinical judgement. - As with all injectable vaccines, appropriate medical treatment and supervision should always be readily available in case of an anaphylactic event following the administration of the vaccine. - As with other vaccines, vaccination with Shingrix should be postponed in subjects suffering from an acute severe febrile illness. However, the presence of a minor infection, such as a cold, should not result in the deferral of vaccination. - As with any vaccine, a protective immune response may not be elicited in all vaccinees. - The vaccine is for prophylactic use only and is not intended for treatment of established clinical disease. - Shingrix should be given with caution to individuals with thrombocytopenia or any coagulation disorder since bleeding may occur following intramuscular administration to these subjects. - Syncope (fainting) can occur following, or even before, any vaccination as a psychogenic response to the needle injection. It is important that procedures are in place to avoid injury from faints. - Shingrix is not indicated for the prevention of primary varicella infection (chickenpox)
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Shingrix powder and suspension for suspension for injection, Herpes zoster vaccine (recombinant, adjuvanted)
Legal category	POM
Storage	Store in a refrigerator (2°C–8°C). Do not freeze. Protect from light.

	After reconstitution chemical and physical in use stability has been demonstrated for 24 hours at 30 °C. From a microbiological point of view, the vaccine should be used immediately. If not used immediately, in use storage times and conditions prior to use are the responsibility of the user and would normally not be longer than 6 hours at 2 °C to 8 °C.
Route/method of administration	- Intramuscular injection, preferably in the deltoid muscle. - The powder and the suspension must be reconstituted prior to administration. - The SmPCs/package inserts should be consulted for preparation steps. - GSKPro also provides a video of the reconstitution steps: gskpro.com/en-gb/products/shingrix/shingrix/overview - Shingrix should not be administered intravascularly or intradermally as subcutaneous administration is not recommended. - Maladministration via the subcutaneous route may lead to an increase in transient local reactions.
Dose and frequency	0.5 mL dose given twice, with the second dose 2 to 6 months after the first.
Quantity to be administered and/or supplied	0.5 mL per dose; two-dose course required
Maximum or minimum treatment period	Two doses required, spaced 2–6 months apart.
Adverse effects	• Localised pain, redness, swelling at injection site, fatigue, headache, myalgia, shivering, fever, gastrointestinal symptoms. Refer to SmPC for full details. • In the event of a person developing a varicella (widespread) or shingles-like (dermatomal) rash post-vaccination a vesicle fluid sample should be sent for analysis to confirm the diagnosis and determine whether the rash is vaccine-associated or wild-type. This service is available at the Virus Reference Department (VRD) at the UK Health Security Agency (UKHSA). Health professionals are requested to obtain vesicle swabs from their local hospital laboratories. Forms and instructions on how to take a vesicle fluid sample can be found at: https://www.gov.uk/government/publications/varicella-zoster-virus-referral-form • Adrenaline injection 1mg/ml (1:1000) must always be available whenever vaccines are given. Immediate treatment for anaphylaxis should include early administration of intramuscular adrenaline and a call to the emergency services. The healthcare professional delivering the service must be trained to recognise an anaphylactic reaction and be familiar with techniques for resuscitation of a patient with anaphylaxis.

	Fainting can occur following, or even before, any vaccination especially in adolescents. It is important that procedures are in place to avoid injury from faints.
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Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: - that valid informed consent was given - name of individual, address, date of birth and GP - name of healthcare practitioner - name and brand of medication - date of supply dose, form and route quantity supplied - advice given, including advice given if excluded or declines treatment - details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: - Adults aged 18 years and over - keep records for 8 years - Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	- To seek medical advice if symptoms worsen rapidly or significantly, do not improve in 3–4 weeks, or they become systemically very unwell. - Inform the patient/carer of possible side effects and their management - Give advice regarding a normal reaction to the injection e.g. redness and pain at the injection site - Advice patient/carer to seek medical advice in the event of a severe adverse reaction

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mg2 - NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources - NICE CKS Guidance - British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines BNF (British National Formulary) NICE - Summary of Product Characteristics for drug
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Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date		Expiry date	
15/2/26		31/10/26	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or vaccine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		15/2/26
Chris Pilkington	Pharmacist GPhC: 2046322		15/2/26

Change history

Version number	Change details	Date
001	Development & issue of new PGD	15/2/26